



Diabetic Retinopathy

Diabetic retinopathy is a complication of diabetes that affects the retina, the light-sensitive tissue at the back of the eye. It occurs when high blood sugar damages the tiny blood vessels inside the retina, potentially leading to vision loss.

Who Is at Risk?

Anyone with Type 1 or Type 2 diabetes is at risk — especially those with:

- Long-standing or poorly controlled diabetes
- High blood pressure
- High cholesterol
- Pregnancy
- Smoking

The risk increases the longer you've had diabetes.

Stages of Diabetic Retinopathy

1. Mild/Moderate Non-Proliferative: Early stages with small areas of leakage or swelling.
2. Severe Non-Proliferative: Blocked blood vessels can lead to areas of poor oxygen supply.
3. Proliferative Diabetic Retinopathy: New abnormal blood vessels grow — these are fragile and may bleed.
4. Diabetic Macular Edema (DME): Fluid buildup in the central retina (macula) causing blurry vision.

Symptoms

You may not notice symptoms in early stages. As it worsens:

- Blurred or fluctuating vision
- Spots or floaters
- Dark areas or vision loss
- Trouble seeing at night

Vision loss can occur without pain, making regular exams critical.

Diagnosis

Diabetic retinopathy is diagnosed with:

- Dilated retinal exam by an eye doctor



- OCT scan (to detect swelling)
- Fluorescein angiography (for detailed vessel imaging)

Treatment Options

Early stages may not require treatment — just tight diabetes control and regular monitoring.

For advanced cases:

- Anti-VEGF injections: reduce swelling and prevent new vessels
- Focal/grid laser: seals leaky vessels
- Panretinal laser (PRP): shrinks abnormal vessels
- Vitrectomy: surgery to remove bleeding or scar tissue

Living with Diabetic Retinopathy

- Keep your blood sugar, blood pressure, and cholesterol under control
- Don't skip annual dilated eye exams — even without symptoms
- Seek prompt care if vision changes
- Talk to your doctor about managing risk factors

Frequently Asked Questions

Q: Can diabetic retinopathy be reversed?

A: Early changes can improve with better diabetes control. More advanced damage may be slowed or stabilized with treatment, but not fully reversed.

Q: Will I go blind?

A: If left untreated, it can cause severe vision loss. But with early detection and treatment, many people maintain good vision.

Q: Can I prevent it?

A: Keeping your diabetes well-controlled, avoiding smoking, and getting annual eye exams are the best ways to prevent progression.